

PATIENT SCREENING REPORT

Retinal Edge Triage — Explainable AI-Assisted Diabetic Retinopathy Screening for Rural Healthcare
Screening Date & Time: 05-Sep-2026 15:37 UTC | Reference ID: SCR-2026-000053

Patient ID: RET-2026-000054	Full Name: Ramesh Gowda	Age / Gender: 58 / Male
Screening ID: SCR-2026-000053	Location: Mysuru Rural PHC	Diabetes Type / Duration: Type 2 (8 years)

1. Retinal Image Acquisition & Quality Gate

Eye Evaluated: Right Eye	Image Source: Portable Fundus Camera	Quality Score: 67.3%	Quality Status: BORDERLINE
Focus / Blur: 31.2%	Illumination: 52.2%	Vessel Visibility: 79.7%	Field of View: 91.5%

2. Diabetic Retinopathy Screening & Detection Result

DR Detection Result:	Diabetic Retinopathy: DETECTED	DR Grade & Severity:	Grade: 2 — Moderate NPDR
Model Confidence:	96.6%	DME Risk Status:	Possible DME detected
Result Explanation:	Moderate non-proliferative changes with microaneurysms (8) and exudates (11). Secondary review indicated.	Recommended Action:	Ophthalmology evaluation recommended within 3 months for stereoscopic examination.

3. Human-in-the-Loop Ophthalmologist Adjudication

Doctor Decision:	ACCEPTED	Reviewing Doctor:	Dr. Ophthalmologist
Final Grade Assigned:	Grade 2	Override Justification:	None
Clinical Notes:	Retinal signs compatible with diabetic retinopathy.	HCW Instructions:	Issue referral and schedule patient transport.

4. Risk Triage & Clinical Disposition

Risk Category:	HIGH RISK	Referral Priority:	URGENT
Referral Reference ID:	REF-2026-00021	Follow-up Schedule:	3 Months
Clinical Justification:	Moderate DR with DME risk identified.	Referral Status:	PENDING

5. Recommended Eye-Care Facility (Referral Destination)

Hospital / Clinic Name:	ABC Eye Hospital	Distance:	2.4 km away
Hospital Address:	Sayyaji Rao Road, Medar Block, Yadavagiri, Mysuru, Karnataka 570020	Contact Number:	+91 821 241 9300

Reason for Referral:	Moderate NPDR requires ophthalmic evaluation based on AI screening result.	Navigation / Directions:	View Map & Directions
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STATUTORY CLINICAL NOTICE: This report is generated by an artificial intelligence screening triage prototype (Retinal Edge Triage). AI predictions DO NOT constitute a definitive medical diagnosis. Final clinical interpretation and management must always be performed by a qualified ophthalmologist. Compliant with rural screening clinical governance principles.